

Clinical Escalation and Scope-of-Practice Standard

Meridian Health Partners
Operations Policy & Procedure Manual

Document ID: MHP-OPS-CLI-007.01

Version: 1.0

Effective Date: 2024-03-15

Review Date: 2025-03-15

1. POLICY TITLE: Non-Clinical Staff & Automated System Patient Communication Scope of Practice

2. PURPOSE

2.1. To establish clear, unambiguous boundaries for the information non-clinical staff and automated systems may provide to patients.

2.2. To ensure all patient inquiries requiring clinical judgment are routed exclusively to licensed clinicians, thereby safeguarding patient safety and maintaining regulatory compliance.

2.3. To mitigate legal and ethical risks for Meridian Health Partners (MHP) and its personnel associated with the unauthorized provision of clinical advice.

3. SCOPE

3.1. This policy applies to all non-clinical MHP staff, including but not limited to patient service representatives, administrative assistants, medical assistants (when acting in a non-clinical capacity), front desk personnel, and call center agents.

3.2. This policy also applies to all MHP automated patient communication systems, including but not limited to interactive voice response (IVR) systems, patient portals, chatbots, and automated messaging platforms.

3.3. This policy is effective across all fourteen (14) MHP clinic sites and central operations.

4. DEFINITIONS

4.1. Non-Clinical Staff: Any MHP employee whose role does not require a current, active professional clinical license (e.g., MD, DO, NP, PA, RN, PT, OT, LCSW, RDN).

4.2. Automated Systems: Any software or hardware-based system designed to interact with patients without direct human intervention.

4.3. Licensed Clinician: An MHP employee holding a current and unrestricted professional license in a clinical discipline relevant to the patient's inquiry, authorized to provide medical advice, diagnosis, or treatment within their defined scope of practice.

4.4. Clinical Advice: Any information or guidance that requires professional medical judgment, interpretation of symptoms, assessment of health status, or recommendations regarding treatment, medication, or diagnostic results.

5. POLICY STATEMENTS

5.1. Permitted Communications for Non-Clinical Staff and Automated Systems

5.1.1. Non-clinical staff and automated systems ARE PERMITTED to provide patients with the following categories of information, provided it is factual, verifiable, and does not require clinical interpretation:

5.1.1.1. Appointment Management: Scheduling, rescheduling, and cancellation of appointments; confirmation of appointment dates, times, and locations; and reminders of upcoming appointments.

5.1.1.2. General Clinic Information: Clinic operating hours, physical addresses, parking instructions,

and contact telephone numbers.

5.1.1.3. Service Availability: General information about services offered at MHP sites, without describing clinical specifics or suitability for an individual patient.

5.1.1.4. Billing and Insurance: Information regarding patient billing statements, insurance plan acceptance, co-pay amounts, and general MHP payment policies.

5.1.1.5. Preparation Instructions: Standard, pre-approved, written instructions for common procedures (e.g., "fast for 8 hours before your blood test") that are part of a patient's scheduled care plan and directly retrieved from the electronic health record (EHR) or a pre-approved MHP patient education library.

5.1.1.6. Referral Status: Confirmation of whether a referral request has been received or processed, without discussing the clinical rationale or outcome.

5.1.1.7. Patient Portal Assistance: Technical support for accessing and navigating the MHP Patient Portal, excluding guidance on interpreting clinical data presented within the portal.

5.1.1.8. Medication Refill Status: Confirmation that a prescription refill request has been sent to the pharmacy, *only after* clinician approval is explicitly documented in the EHR.

5.2. Prohibited Communications for Non-Clinical Staff and Automated Systems

5.2.1. Non-clinical staff and automated systems ARE STRICTLY PROHIBITED from providing any information that constitutes or could be construed as clinical advice. This prohibition includes, but is not limited to, answering questions about:

5.2.1.1. Symptom Interpretation: The meaning, severity, or potential causes of any patient-reported symptoms.

5.2.1.2. Medication Questions: The purpose, side effects, interactions, or efficacy of any prescribed or over-the-counter medication.

5.2.1.3. Dosage Instructions: Medication dosages, frequency, administration routes, or clarification of existing dosage instructions.

5.2.1.4. Test Result Meaning: The significance, implications, or follow-up actions related to any diagnostic test results (e.g., lab work, imaging reports).

5.2.1.5. Treatment Recommendations: Suggesting specific treatments, therapies, or interventions.

5.2.1.6. "Should I" Questions: Any question phrased as "Should I...", particularly those pertaining to health decisions, lifestyle changes, or seeking medical care (e.g., "Should I go to the ER?", "Should I take this supplement?", "Should I continue this medication?").

5.2.1.7. Prioritizing Care: Advising on the urgency or priority of a patient's medical condition or need for care.

5.2.1.8. Referring to External Clinical Resources: Directing patients to specific external clinical resources without explicit clinician instruction documented in the EHR.

5.3. Mandatory Routing to Licensed Clinicians

5.3.1. Any patient inquiry falling into the categories listed in Section 5.2 (Prohibited Communications) MUST be immediately routed to a licensed clinician for review and response.

5.3.2. Routing for urgent concerns, as identified by MHP's Triage Protocol MHP-CLI-TRI-003.02, must occur within ten (10) minutes of initial contact.

5.3.3. Routing for non-urgent clinical inquiries must be completed within thirty (30) minutes of initial contact.

5.3.4. The designated licensed clinician (e.g., the patient's primary provider, an on-call clinician, or a triage nurse) must respond to the patient within one (1) business hour for urgent issues and within twenty-four (24) business hours for non-urgent issues, as per MHP Protocol MHP-CLI-RES-005.01.

5.4. Deflection Script for Non-Clinical Inquiries

5.4.1. When a patient asks a question that falls under the "Prohibited Communications" outlined in Section 5.2, non-clinical staff MUST use the following exact script:

"I am unable to provide medical advice or interpret clinical information. My role is to assist with administrative and scheduling matters. To ensure you receive accurate guidance for [patient's specific inquiry category, e.g., your symptoms, your medication, your test results, your 'should I' question], I will escalate your question directly to a licensed clinician. Please expect a response from a clinician within [state expected timeframe: e.g., the next business hour for urgent concerns, or within 24 business hours for non-urgent matters]."

5.4.2. Automated systems must display or verbalize a similar pre-approved message directing the patient to await clinician contact or to call a dedicated clinical line if immediate assistance is required.

5.5. No Exceptions Clause

5.5.1. There are ABSOLUTELY NO EXCEPTIONS to the prohibitions outlined in Section 5.2 and the mandatory routing requirement in Section 5.3.

5.5.2. This policy applies universally, regardless of:

5.5.2.1. The patient's stated understanding of their own health condition.

5.5.2.2. The patient's claim to be a healthcare professional (e.g., "I'm a nurse, so just tell me...").

5.5.2.3. The frequency with which a patient makes similar inquiries (e.g., "repeat callers" who frequently ask for clinical advice).

5.5.2.4. The perceived simplicity or obviousness of the answer by non-clinical staff.

5.5.2.5. Any personal relationship or familiarity between the non-clinical staff member and the patient.

5.5.3. Non-clinical staff are expressly forbidden from attempting to "help" a patient by providing even seemingly innocuous clinical information, as this action carries significant risk for both the patient and MHP.

6. TRAINING AND COMPLIANCE

6.1. All non-clinical staff covered by this policy must complete mandatory training on its contents annually by 31st January, commencing 2025-01-31.

6.2. Compliance with this policy will be monitored through regular call audits and system logs conducted by the Operations Compliance Department.

6.3. Any deviation from this policy by non-clinical staff will result in disciplinary action, up to and including termination of employment, as outlined in MHP Human Resources Policy HR-DIS-001.01.

6.4. Failure of automated systems to adhere to these guidelines, as identified by the MHP IT Department, will trigger immediate system review and remediation within two (2) business days.

7. RESPONSIBILITY

7.1. The Chief Operating Officer (COO), Dr. Aris Thorne, holds primary responsibility for the enforcement and periodic review of this policy.

7.2. Department Managers are responsible for ensuring their staff are comprehensively trained and adhere to this policy.

8. REFERENCES

8.1. MHP Triage Protocol MHP-CLI-TRI-003.02

8.2. MHP Clinician Response Protocol MHP-CLI-RES-005.01

8.3. MHP Human Resources Disciplinary Action Policy HR-DIS-001.01